جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : رقيه راشد عمر سيد
المحافظة : القاهره **الحي / المدينة :** بولاق ابوالعلا **العنوان :** بولاق
رقم البطاقة : 27902182601981 **رقم التليفون :** 01007772701 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 18-02-1979 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبطات الاروماتيز [14161] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رقيه راشد عمر سيد
National ID: 27902182601981

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
نوبار
نوميا



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Ruqayyah Rashid Omar Said . Date : 25-05-2026 10:00
Branch : Dokki Medical No. : 382228
Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP Whole-Body Skeletal Scintigraphy

Clinical History

A 47-year-old female patient with a known history of right breast carcinoma (undergoing systemic hormonal and targeted therapy) presents for follow-up evaluation. The patient reports no acute localized bony pain or specific skeletal symptoms at present.
Prior radionuclide bone scan dated 15/12/2025 demonstrated a regressive course of widespread skeletal metastases compared to a historical baseline from 09/07/2025.

Technique

Following the intravenous administration of Tc-99m MDP, anterior and posterior whole-body planar scans, were acquired approximately 2 hours post-injection.

Findings

Metastatic Disease Assessment:

- **Axial Skeleton:** Persistent, low-grade, faint tracer uptake noted at the skull vault, multiple spinal levels, sternum, ribs, and pelvic bones. No new or highly intense osteoblastic lesions are seen.
- **Appendicular Skeleton:** Symmetric, near-physiological tracer distribution in both scapulae, humeri, and proximal femora.

Degenerative / Benign Alterations:

- Mild-to-moderate uptake at the large weight-bearing joints (bilateral knees, tarsi) and lower lumbar facets, consistent with degenerative arthropathy.

Comparison

- Compared with the prior study dated **15/12/2025**, there is complete stability to further regression of the previously reported widespread metastatic skeletal lesions. No new active foci or signs of disease progression.

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Name: راقية راشد عمر **MR. :** 245	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

premenopausal rt breast cancer metastatic to bone ,med ln er7/8.pr8/8,her2 3+ rec 8 cycles taxotere and herceptin st ai and zoladex from 4/2025 bs on 12/2025 RD CTS 11/2025;SD for herceptin ai zometa

EF30/12/2025 66% FOR HERCEPTIN IV 414 MG WT69 ARIMIDEX,ZOLADEX DUE DATE1/3

FOR HERCEPTIN IV 414 + ARAMIDEX + ZOLADEX DUE TODAY EF30/12/2025 66% NEW ECHO NEXT TIME NEXT TWO CYCLES PHARMACY WITH AI

FOR HERCEPTIN IV 414 + ARAMIDEX + ZOLADEX DUE TODAY EF 65%18/3/2026 NEXT TWO CYCLES PHARMACY WITH AI

PHARMACY VISIT:FOR HERCEPTIN IV 414 + ARAMIDEX EF 65%18/3/2026

bone scan 25/5/2026 stable to further regressive course of known widespread skeletal metastasis . cts on 24/5/2026 sd of rt breast mass rt ax ln ,subcentimetric paraortic ln ,multiple vertebral lytic lesions for bs for zoladex herceptin 415(wt69) arimidex for dental ex for zometa next time

CT CAP 24-5-2026 compared to 2-11-2026 >> SD bone scan 25-5-2026 compared to 9-7-2025 >> SD further regressive disease

for herceptin IV + Arimidex + zometa pt did dental exam > approved echo 22/7 65% EF (labs pending)

for herceptin 415 arimidex . zoladex due date 20/9 (wt 690 for new cts

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Ruqayyah Rashid Omar Said | Date : 25-05-2026 10:00
Branch : Dokki | Medical No. : 382228
Ref.Physician Prof : Goustav Roussey Egypt

Impression

- Stable to further regressive course of known widespread skeletal metastases. No scintigraphic evidence of active or progressive disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

EGYPT - 6 OCTOBER - 26TH OF JULY CORRIDOR - KM 2 - ALEXANDRIA DESERT ROAD

Call Center 19656

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient's Name:	**Ruqayya Rashid Omar**
Date:	**2026-05-24**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

CECT CHEST:

****Compared to previous study dated on 2-11-2025:**

- **Stationary course as regard,**
- Right breast shows mild diffuse skin thickening, parenchymal asymmetry and non suspicious right axillary lymph nodes.
- Multiple right upper, middle and bilateral lower lung nodules, largest one is seen at right middle lung lobe pleural based and still measuring 9 mm.

- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Multiple paraaortic enlarged sub centimetric lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows multilevel vertebrae lytic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Ruqayya Rashid Omar**
Date:	**2026-05-24**
Referred by:	**Prof.Dr. no doctor**
Hospital:	**Goustave Roussey International Egypt**

OPINION:

- **Stationary course as regard chest findings compared to the previous attached study dated on 2-11-2025:**
- **The scanned skeleton shows multilevel vertebrae lytic lesions...For bone scan assessment.**
- **Multiple paraaortic enlarged sub centimetric lymph nodes.**

Dr.Heba El Balshy.MD
Lecturer and Consultant of Diagnostic and Interventional Radiology,
NLI-Menoufia University

CT Scan
[[DOCTYPE:CT_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : رقيه راشد عمر سيد
Sex: Female
3- Medical number: 83211267
Old Med : 1786-2024

2-ID number: 27902182601981
Patient Age : 45 Y

MDT

Family history	Family History -ve	Phone	Phone 01016177341	BMI	BMI	REASON OF MDT	REASON OF MDT NEW
marital status	Marital Status MARRID	NRS Score	NRS Score	GROUP	GROUP M	Date of MDT	Date Of MDT 14/07/2024
Co morbidity	Co Morbidity dm,htn	Menopausal status	Menopausal Status PERIMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
locally advanced

Mets Workup :
pet ct:MBC to bones with high uptake over rt femur & pelvis

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	7/8	8/8	3		
BX COMMENT: gil	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M
		bone

Sonomammography

PATIENT HISTORY
pet ct:MBC to bones with high uptake over rt femur & pelvis

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
* Start Taxotere , Herceptin

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY د/ احمد زايد
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / رقيه راشد عمر سيد	Request No.	: 13004589
Patient No.	: 27902182601981	Request Date	: 08/06/2024
Path No.	: SR-745/24	Verification Date	: 23/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:
- One paraffin block labeled by (200884/24) and three slides were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Minimal component (commedo).
* Angioinvasion: No.
* Perineural invasion: No..
* IPT:
- ER: Moderate nuclear reaction in 80% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:
Right breast cancer, Slide revision:
Invasive duct carcinoma, Grade II with minimal DCIS.
IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).

Signature
Dr. Walaa Ghonim.
Dr. Heba Hassan.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / رقيه راشد عمر سيد	Request No.	: 13004589
Patient No.	: 27902182601981	Request Date	: 08/06/2024
Path No.	: SR-745/24	Verification Date	: 23/06/2024
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Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Verified by
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Rokaya Rashed Omar Sayed	Date : 19/03/2025
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 1786/2024	Pat. ID: 95180

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

In comparison to previous contrast study dated at 8-12-2024

Findings:

I Chest:

- **Rather Stationary course** regarding previously noted:
  - Right breast skin thickening & parenchymal asymmetry as well as the Few sub cm right axillary lymph nodes.
  - Number and size of the few small and tiny middle, right upper and bilateral lower pulmonary ndouels, the largest seen at middle lobe pleural / fissural based and still measures 0.9 cm.
- No pulmonary consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Left thyroid lobe hypodense nodules and calcifications for US.

II Abdomen & pelvis:

- The liver is enlaregd with homogenous low CT texture (fatty liver). No definte focal lesions or contour irregularities on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Rokaya Rashed Omar Sayed	Date : 19/03/2025
Exam : C.T CHEST	Age : 46 Year(s)
Medical ID: 1786/2024	Pat. ID: 95180

- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- The axial and appendicular skeletion shows rather stationary course regarding previously noted innmerable mixed lytic and sclerotic deposits .

Opinion:

- **Rather stationary course since the last study**
- **No evidence of definite hepatic metastatic deposits on non-contrast basis.**

Dr. Ahmed Elgazar

Dr. Walid Mohamed, M.D

CT Scan
[[DOCTYPE:CT_SCAN]]

Oncology Center
مركز الأورام

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

خطابات أورام

Nuclear Medicine Unit

Patient Name: Rokiah Rashed Omar ***Date: 18/2/2025***

Referring physician: Prof.

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- Widely distributed axial ,appendicular skeleton , marrow based active metastatic osseous lesions , distributed at , the sternum , both proximal humerei , both scapulae , left clavicle , multiple bilateral ribs , most of dorso-lumbar vertebrae , sacrum , multiple pelvic bones , including both sacro-iliacs , Rt femur , (head , neck , trochanter, proximal shaft), for follow up .
- Both kidneys are normally seen.

Conclusion:

Wide spread, axial , appendicular skeleton , marrow based active osseous metastatic lesions as mentioned above , for follow up .

Nuclear Medicine Unit

DR . Sherin Mahdy, MD

DR Mohamed Emam

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦

1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel.: 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Name : Ruqayyah Rashid Omar Said	Date : 09-07-2025 02:51
Branch : Maadi	Medical No. : 147091
Ref.Physician Prof : Hrml Hospital	Ins.Company : التامين الصحي

NILE DIAGNOSTIC CENTER

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- A 46-year-old lady, with history of right breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

The previous bone scan images dating 18/2/2025 are not available for comparison

- Abnormal tracer uptake with heterogenous pattern is seen at proximal end of the right humerus, both humeral shafts, sternum, multiple bilateral ribs, multilevel of the spine, right sacroiliac region as well as trochanteric region of the right femur.

Impression:

- Picture of widespread metastatic skeletal deposits as described above.
- Follow-up bone scan is recommended to monitor therapy response.

Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

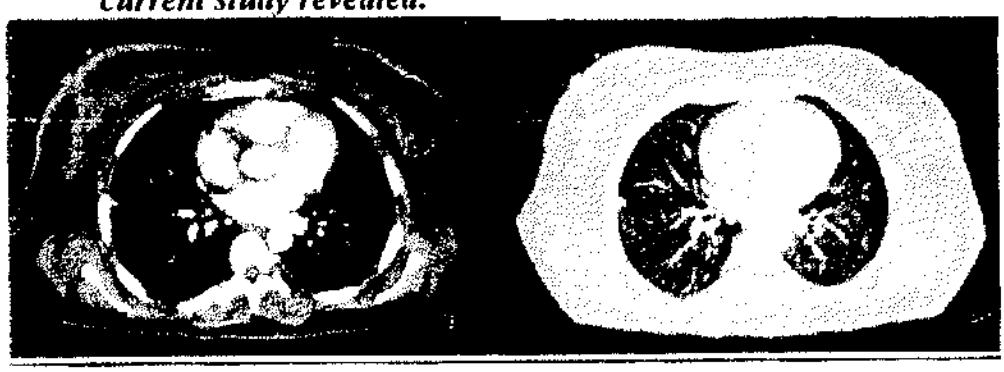
***Name :* Roukaya Rashed**

***Date :* 20-7-2025**

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: - PRIMARY REPORT

Clinical History:

- ***Known case of breast cancer, compared to the previous study report, the current study revealed:***



Findings:

- **Rather Stationary course** regarding previously noted:
  - Right breast skin thickening (11.5 mm) and parenchymal asymmetry as well as the few subcentimetric right axillary lymph nodes.
  - Number and size of the few small and tiny middle, right upper and bilateral lower pulmonary nodules, the largest still seen subpleural at the middle lobe still measures 9 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural or pericardial effusion.
- Average size of the heart.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Still noted left thyroid lobe hypodense nodules and calcifications for US.

OPINION:

- ***Stationary course of the disease process.***

Marina Essam, MSc

CT Scan
[[DOCTYPE:CT_SCAN]]

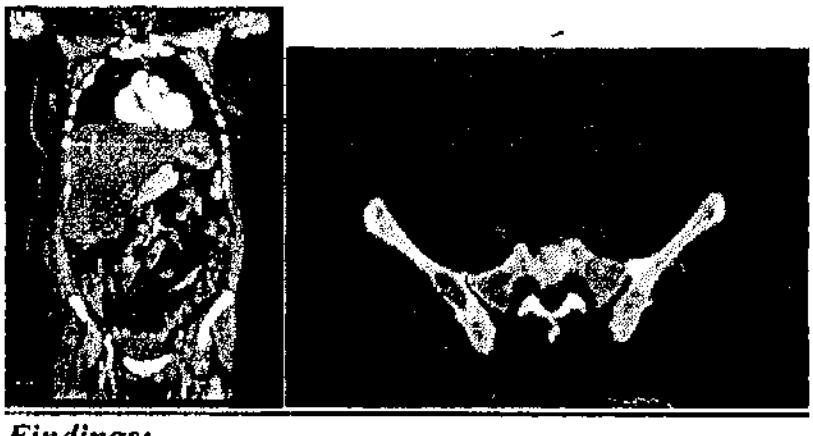
Name : ***Roukaya Rashed***

Date : ***20-7-2025***

MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: - PRIMARY REPORT

Clinical History:

- ***Known case of breast cancer. Compared to the previous study report, the current study revealed:***



Findings:

- The liver is enlarged (17 cm) with low homogenous CT texture (fatty infiltration). No sizable focal lesions.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size (12.5 cm) and parenchymal texture of the spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.
- The scanned bones still show innmerable mixed lytic and sclerotic deposits.

Opinion:

- ***Fatty hepatomegaly for clinical and lab correlation.***
- ***Still noted mixed lytic ad sclerotic osseous deposits.***

CT Scan
[[DOCTYPE:CT_SCAN]]